PwC Digital Strategy

Stakeholder Interview Analysis

Proposed healthcare acquisition for new-market entry

Prepared for: Client executive committee (pre-diligence discussion)

Date: 31 October 2026

Status: Draft for discussion

Important basis of preparation

This report draws only on eight fictional practice interview transcripts, each of which states that its statements "require validation". The transcripts do not identify the target, its business model, its countries of operation, its products, or any financial, regulatory or quality data. No claim in this report has been externally verified, and stakeholder views are presented as opinions, not as organisational facts. This report does not make an acquisition recommendation.

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1. Executive summary

We analysed eight stakeholder interviews covering strategy, market access, clinical quality, regulation, health data and cyber, finance, operations and local market conditions. The picture is consistent across them. The interviewees do not report findings about the target. They set out what must be proven before the decision, and most of that proof is still missing.

What the interviews tell us

- The strategic case rests on speed, but the Chief Strategy Officer states that the team has "not yet shown" which advantages the target genuinely owns, or whether a partnership could achieve the same goal.
- Several stakeholders (S2, S8) caution that a large market is not an accessible one, and that relationships and reputation may not transfer across borders or survive a change of control.
- Patient safety, regulatory permission, health-data protection and service continuity are each described as gating issues. Each stakeholder lists red flags that would need escalation, but none reports that a red flag exists.
- The CFO asks for independent quality-of-earnings work, a costed synergy register, sensitivity analysis and client-supplied investment thresholds, and notes that the recommendation may need to be conditional or paused if critical assumptions cannot be validated.

What the interviews do not tell us

The transcripts do not establish what the target is, where it operates, what it sells, its regulatory standing, its safety record, its data practices, its supply dependencies or its financial performance. All of this is "Not established in transcript."

Implication

These interviews support defining and sequencing the evidence needed for due diligence. They do not support, and this report does not make, an acquisition recommendation. Section 10 sets out what the interviews imply for the decision, and Section 11 proposes next steps.

2. Scope, sources and method

Source: eight interview transcripts (one per stakeholder listed in Appendix B), each labelled "Fictional practice transcript created for the hackathon; statements require validation." No other source was used. We did not use external research, and no statistics, countries, products or target details have been added.

Every item in this report is labelled with one of the following evidence classifications.

Label	Meaning
Transcript statement	What a stakeholder said, requested or asked. This records that it was said; it does not confirm it is true.
Stakeholder opinion	A view or judgement expressed by a stakeholder. Not a verified organisational fact.
Hypothesis	A proposition that is untested, whether stated by a stakeholder or describing an assumption the deal team might hold.
Consultant inference	Our own reading across interviews. It should be tested, and is not something stakeholders said.
Externally verified fact	None. No statement in the transcripts has been externally verified.

Quotations are verbatim excerpts from the transcripts. Where only part of a sentence is quoted, the excerpt is shown unaltered, and paraphrases are our wording. Where the transcripts do not answer a question, we write "Not established in transcript."

3. Strategic priorities

The priorities below are those expressed or implied by the interviewees. They are stakeholder views and are not agreed corporate priorities. The transcripts do not state a client-approved investment thesis.

Priority	Raised by	Evidence status	Gap
Establish what the target genuinely owns (local access, relationships, regulatory know-how, distribution, specialist capability) and whether a partnership or organic entry could achieve the same aim.	S1	Stakeholder opinion; gap acknowledged by S1	Not established in transcript
Test the specific market segment: size, growth, purchasing process, decision-maker, alternatives, adoption barriers.	S1, S2, S8	Stakeholder opinion	Not established in transcript
Define measurable strategic outcomes and avoid double-counting revenue growth and synergy.	S1	Stakeholder opinion	Outcomes and metrics not established in transcript
Confirm regulatory permission to operate in each target country and model the approval pathway in the forecast.	S4, S8	Stakeholder opinion	Target products, countries and licences not established in transcript
Verify clinical value, quality and patient-safety evidence independently before commitment.	S3	Stakeholder opinion	Not established in transcript
Maintain safe service and product continuity through integration, with a Day 1 readiness package.	S3, S7	Stakeholder opinion	Not established in transcript
Understand health-data, cyber and system-integration exposure and cost them.	S5	Stakeholder opinion	Not established in transcript
Validate revenue quality, true all-in cost and downside cash-flow impact.	S6	Stakeholder opinion	Financial data not established in transcript
Replace management optimism with customer evidence of a credible route to adoption.	S2, S8	Stakeholder opinion	Not established in transcript

4. Healthcare-specific risk assessment

The table sets out the risk areas the interviewees raised. In every case the stakeholders describe what should be examined, not what has been found. The "Status" column therefore records the state of the evidence, not a risk rating. We have not assigned severity or likelihood because the transcripts contain no target data to support them.

Risk area	What stakeholders raised	Raised by	Status	Evidence needed
Patient safety / quality	Complaints, adverse events, recalls, repeat defects, CAPA closure; how "high quality" is measured and verified; continuity of care or product availability under change of ownership, supplier, system or process.	S3, S7	Questions and evidence requests only. No safety or quality findings are reported.	Complaint, incident, recall and audit records; independent quality review; critical products/services map.
Regulatory permissions	Licences, registrations, facility approvals, change-of-control notifications and local representation by country; inspection and regulator history; timing and cost of missing approvals.	S4, S8	Questions and red-flag criteria only. The target's regulatory status is not established in transcript.	Product/service and country list; licence register; regulator correspondence; legal and regulatory review.

Risk area	What stakeholders raised	Raised by	Status	Evidence needed
Market access / tendering	Established supplier relationships, tender rules, switching costs, evidence requirements, local trust; whether relationships transfer across borders and survive a change of control.	S2, S8	Opinion and hypotheses. No market-access data is provided.	Customer-level contracts including change-of-control terms; lost/won bids; customer references; decision-chain map.
Reimbursement / payer exposure	Reimbursement, budget constraints and price pressure "where applicable"; payer mix, rates and denials if payer exposure exists.	S2, S6, S8	Conditional. Whether the target has payer or reimbursement exposure is not established in transcript.	Payer mix, reimbursement rates, denial patterns, contract terms (if applicable).
Health data / cybersecurity	Data collected, stored, shared across borders or with vendors; legal grounds; incident and breach history; migration and interoperability risks to records and workflows; AI opportunity treated as unproven.	S5	Questions and evidence requests only. The target's data practices are not established in transcript.	System and data-flow inventory; penetration-test findings; backup/recovery tests; breach history; data-rights analysis.
Supply / service continuity	Single-source suppliers, lead times, cold chain, traceability, capacity, staffing, escalation and authority to stop an unsafe change.	S3, S7	Questions and evidence requests only. The target's supply chain and capacity are not established in transcript.	Critical-operations map; supplier dependency list; OTIF, stockout, downtime and capacity data; continuity plan.
Financial sustainability	Revenue reconciliation, concentration, renewals; deferred capex, remediation, cyber upgrades, working capital, retention and integration costs; downside cash flow and returns.	S6, S2	Opinion and evidence requests. No financial figures are in the transcript.	Independent quality-of-earnings work; reconciled historical financials; costed synergy register; sensitivity analysis; client investment thresholds.

5. Evidence cited and requested

No interviewee cites existing evidence about the target. Each instead requests evidence. Several stakeholders (S2, S3, S4, S5, S6) explicitly ask for original or independently verified records in place of management assertion. The requests are summarised below.

Stakeholder	Evidence requested
S1	Source-traced sector scan; target-specific competitor and market map; validated customer evidence; realistic entry timeline; assessment of alternatives; downside case. S1 asks that "what we know, what management claims, and what remains untested" be shown separately.
S2	Customer-level revenue, concentration, contract duration and renewal dates, lost and won bids, pipeline conversion, pricing history, complaints, distributor/channel agreements, reasons for switching; payer mix, rates, denials and contract terms if relevant; customer references and independent sources.
S3	Clinical studies, real-world outcomes, safety and performance data, quality indicators, patient experience; complaints, incidents, recalls, CAPA and audit findings; definition, period, comparator and independent verification of any quality metric; independent review.
S4	List of products, services and countries; licence, registration and approval map; inspection reports, regulator correspondence, sanctions, licence conditions, pending applications, remediation evidence; originals to reconcile with management answers.
S5	System and data-flow inventory; access-control design; incident and breach history; penetration-test findings; patching status; backup and recovery tests; third-party dependencies; remediation evidence; consent or other legal grounds.
S6	Independent quality-of-earnings work; reconciled historical financials; transparent forecast; costed synergy register; sensitivity analysis; client-supplied investment thresholds; payer, product or facility metrics depending on the business model.

Stakeholder	Evidence requested
S7	OTIF delivery, stockouts, service availability, backlog, downtime, complaints, quality incidents, capacity utilisation, vacancy and turnover, supplier performance, site-level costs; critical-operations map; continuity plan; supplier and service dependency list.
S8	Customer references; contract durability; service coverage; local talent and distribution reach; regulatory experience; direct customer and partner feedback on why they choose the target and whether ownership change would matter.

6. Assumptions and hypotheses

The following propositions underlie the acquisition thesis or were raised as risks to it. None is verified.

ID	Assumption / hypothesis	Source	Status	Test
H1	Acquisition delivers market-entry speed and access that organic entry or partnership would not.	S1	Hypothesis. S1 states it is not yet shown.	Evidence of exclusivity and ownership of advantages; comparison of alternatives.
H2	Target relationships and reputation transfer across borders.	S2, S8 (as a risk)	Hypothesis described as a risk of the deal thesis.	Contract terms, customer references, local reputation evidence.
H3	Existing sales force can cross-sell immediately.	S2 (as a risk)	Hypothesis described as a possible team assumption. Not shown to be held by anyone.	Fit with local purchasing and care pathways.
H4	Relationships are institutional rather than dependent on a founder or a few individuals.	S8	Hypothesis, unverified.	Retention of key individuals; contract holders.
H5	The target can safely scale to meet forecast demand.	S7	Hypothesis, unverified.	Capacity, workforce and supplier data.
H6	A launch date can be planned for each country.	S4	S4 warns against assuming a date. No date is stated.	Regulatory pathway and timeline by country.
H7	AI creates value for the combined business.	S5	Hypothesis by S5's own description. No use case stated.	Data rights, workflow fit, safety, oversight, measurable benefit.
H8	Reported revenue and quality figures are reliable.	S3, S6	Not accepted by S3 or S6 without independent verification.	Quality-of-earnings work; independent quality data review.

7. Cross-interview tensions

We compared the eight interviews for contradictions. We found none that are direct. The items below are tensions or open questions that arise from combining the interviews, and they are our inferences.

ID	Topic	Description	Classification	Test
T1	Speed rationale vs. delay factors	S1 gives speed as the attraction. S2, S4, S7 and S8 each describe approval, procurement, tender, onboarding or integration constraints that could slow benefit realisation.	Consultant inference. Not a direct contradiction; the transcript contains no timelines.	Timeline comparison of acquisition, partnership and organic entry.
T2	Integration pace vs. continuity	S7 and S3 warn that rapid change of suppliers, systems, processes or leadership can disrupt safe operations. S1 emphasises speed of access; no integration plan is described.	Consultant inference.	Integration roadmap with stop-authority and stable-state definitions.

ID	Topic	Description	Classification	Test
T3	Different pause or gate criteria	S1 (non-exclusive advantage, uncertain access), S3 (safety/quality), S4 (licence, conduct, remediation), S6 (unvalidated assumptions) each set separate triggers. Whether they share thresholds is unknown.	Consultant inference.	A single agreed decision-gate framework and client thresholds.
T4	Direct contradictions	No interviewee directly contradicts another. The interviews are broadly aligned in asking for independent, source-traced evidence over management assertion.	Consultant inference from comparison of the eight transcripts.	Re-test once management and customer evidence is gathered.

8. Decision gates stated by stakeholders

Four stakeholders set conditions under which they would pause, escalate or qualify the decision. These are stakeholder opinions and not agreed client criteria. Whether any condition currently applies is Not established in transcript.

Stakeholder	Stated gate or trigger
S1	Rationale rests on a non-exclusive advantage; approvals or access uncertain; reputation does not transfer; acquisition not shown to be better than partnership or organic entry.
S3	Unresolved material safety concern; repeated serious quality finding; unreliable clinical claims; uncosted remediation plan.
S4	Unclear licence or registration status; material undisclosed regulator correspondence; serious unresolved quality findings; evidence of improper conduct; remediation with no credible owner, cost or timeline.
S6	Critical assumptions that cannot be validated may make the recommendation conditional or lead to a pause.

9. Unresolved questions

- What the target is and does (care provider, medicine or device, health-tech platform), and in which countries. Not established in transcript.
- Which advantages the target genuinely owns, and whether they are exclusive. S1 states this has not yet been shown.
- Whether partnership or organic entry could meet the objective, and how the timelines and costs compare.
- Size, growth and accessibility of the specific segments, and who makes the buying decision.
- Licence, registration and approval status by country, and any inspection, sanction or remediation history.
- Safety, quality and complaint history, and how management measures and verifies quality.
- Whether the target has payer or reimbursement exposure, and what its revenue concentration and contract terms look like.
- Health-data practices, cyber posture and the criticality of systems to safe service.
- Supplier concentration, capacity and continuity arrangements.
- Financial results, valuation, one-off and integration costs, and the client's investment thresholds.
- Whether key customer relationships depend on specific individuals and would survive a change of ownership.

10. Implications for the acquisition decision

We make no acquisition recommendation from these interviews. They are a single, unvalidated source, and the transcripts contain no target-specific information. The implications below are our inferences.

- **The thesis is not yet testable.** Without the target's business model, countries and products, the market, regulatory and clinical questions cannot be answered. The first priority is to fix these facts.
- **Evidence must be traceable.** Stakeholders consistently ask for original records, independent verification and customer evidence rather than management claims. The diligence plan should separate what is known, what management claims and what is untested.
- **Several issues could change the decision.** Safety, licensing, conduct, data protection, continuity and unvalidated financial assumptions were each described as gating issues. They should be investigated early, before significant spend.
- **Value and timing need modelling, not assumption.** The stakeholders ask for a regulatory pathway in the forecast, downside cases with cash-flow impact, a costed synergy register and explicit client thresholds.
- **Alternatives remain open.** The CSO asks that acquisition be compared against partnership and organic entry. That comparison has not been done.

11. Proposed next steps

These steps are our suggestions and are for discussion with the client.

#	Action	Owner (proposed)
1	Obtain the target's business model, country footprint and product/service list so every interview point can be mapped to a specific context.	Deal team / client
2	Issue a targeted data request covering the evidence listed in Section 5, prioritising regulatory, quality, contract and financial records.	Deal team
3	Interview target management, customers, local partners, independent quality reviewers and legal specialists, as named by interviewees.	Deal team
4	Agree a single decision-gate framework and obtain the client's investment thresholds.	Client / PwC
5	Build an evidence register that separates verified facts, management claims and untested assumptions, as S1 asks.	PwC
6	Reassess the findings of this report once the evidence in steps 1 to 3 is received.	PwC

Appendix A: Statement register

Exact quotations are verbatim from the transcripts. Paraphrases are our wording. Classification follows the key in Section 2. No statement is externally verified.

ID	Topic	Paraphrase	Exact quote	Stakeholder	Classification	Missing information
ST-01	Strategic rationale	Speed is the main reason to buy rather than build; the advantages the target owns are unproven.	"The attraction is speed." / "we have not yet shown which of those advantages the target genuinely owns or whether we could achieve the same objective through a partnership."	S1	Stakeholder opinion; transcript statement (gap)	Which advantages the target owns; speed gain; partnership alternative. Not established in transcript.
ST-02	Market assumptions	A generic growth claim cannot justify the target or its valuation.	"A broad statement that healthcare demand is growing is not enough to justify this target or its valuation."	S1	Stakeholder opinion	Segments, size, growth, decision-maker, barriers. Not established in transcript.
ST-03	Strategic value / synergy	Define measurable outcomes; avoid double-counting.	"avoid counting the same benefit under both revenue growth and synergy."	S1	Stakeholder opinion	Outcomes and metrics. Not established in transcript.
ST-04	Deal pause triggers	Pause if advantage is non-exclusive, access uncertain, or acquisition not justified over alternatives.	"If the rationale depends on a market-entry advantage that is not exclusive"	S1	Stakeholder opinion	Whether any trigger applies. Not established in transcript.
ST-05	Evidence requested (strategy)	Source-traced evidence pack separating fact, claim and untested items.	"A source-traced sector scan, a target-specific competitor and market map, validated customer evidence, a realistic entry timeline, an assessment of alternatives, and a downside case."	S1	Transcript statement (request)	Owner, sources, timing.
ST-06	Adoption decision chain	Buyers and influencers depend on the business model and must be mapped.	"We need to map the actual decision chain for this target."	S2	Stakeholder opinion	Target business model and decision chain. Not established in transcript.
ST-07	Market access barriers	Growth does not convert to revenue because of incumbents, tenders, reimbursement, evidence needs, switching costs and trust.	"Market growth does not automatically become revenue for one company."	S2	Stakeholder opinion	Which barriers apply by country and segment. Not established in transcript.
ST-08	Competitive position test	Test against competitors and substitutes using customer references and independent sources.	"rather than relying only on the target's pitch deck."	S2	Stakeholder opinion	Competitor set, references, independent sources. Not established in transcript.
ST-09	Commercial data request	A detailed list of customer, bid, pricing and channel data; payer data if relevant.	"Customer-level revenue, concentration, contract duration and renewal dates, lost and won bids, pipeline conversion, pricing history, complaints, distributor or channel agreements, and reasons customers switch."	S2	Transcript statement (request)	None of this data is in the transcript. Payer exposure unknown.
ST-10	Thesis assumptions at risk	The team may assume relationships travel across borders and cross-selling is immediate.	"The team may assume that existing relationships automatically travel across borders or that our current sales force can cross-sell immediately."	S2	Hypothesis (about team assumptions)	Change-of-control terms, retention risk, fit with local pathways. Not established in transcript.

ID	Topic	Paraphrase	Exact quote	Stakeholder	Classification	Missing information
ST-11	Clinical value evidence	Marketing material and testimonials do not establish clinical benefit.	"Marketing material and testimonials alone do not establish clinical benefit."	S3	Stakeholder opinion	Actual clinical or performance data. Not established in transcript.
ST-12	Patient safety signals	Review complaints, incidents, recalls, defects and CAPA closure.	"Complaints, adverse events or safety incidents where applicable, recalls or field actions, repeat defects"	S3	Transcript statement (request)	Actual safety and quality history. Not established in transcript.
ST-13	Continuity of care / availability	Changes in ownership, supplier, system, site or process can disrupt continuity.	"Yes. A change in ownership, supplier, system, service location, or clinical process can affect availability or continuity."	S3	Stakeholder opinion	Critical products/services, dependencies, escalation plan. Not established in transcript.
ST-14	Testing a "high quality" claim	Quality claims need definition, period, comparator and independent verification.	"how quality is measured, over what period, against which comparator, and who independently verified the data"	S3	Transcript statement (questions)	Whether management has made such a claim; any quality data. Not established in transcript.
ST-15	Clinical decision gates	Unresolved safety concerns and similar items should be escalated before commitment.	"Any unresolved material safety concern, repeated serious quality finding, unreliable clinical claims, or uncosted remediation plan should be escalated before commitment."	S3	Stakeholder opinion	Whether any gate item exists. Not established in transcript.
ST-16	Regulatory permission	Map licences and approvals by country; do not assume transferability.	"We must not assume one country's approval works everywhere."	S4	Stakeholder opinion	Products, countries, licences, time and cost of gaps. Not established in transcript.
ST-17	Regulatory history	Review inspections, correspondence, sanctions and remediation against original records.	"We should reconcile management's answers with original records."	S4	Transcript statement (request)	The target's regulatory history. Not established in transcript.
ST-18	Compliance exposure	Applicable rules vary by business and jurisdiction.	"Legal specialists should determine which rules apply in each jurisdiction."	S4	Stakeholder opinion	Which risks apply; any conduct findings. Not established in transcript.
ST-19	Regulatory effect on forecast	Approval timelines and restrictions can delay launch or raise cost.	"The forecast should model the relevant pathway, not simply assume a launch date."	S4	Stakeholder opinion	Forecast, launch date, pathway. Not established in transcript.
ST-20	Regulatory red flags	Listed issues need legal and regulatory review before proceeding.	"An unclear licence or registration status, material undisclosed regulator correspondence, serious unresolved quality findings, evidence of improper conduct, or a remediation requirement with no credible owner, cost, or timeline."	S4	Stakeholder opinion	Whether any red flag exists. Not established in transcript.
ST-21	Critical vs administrative systems	Sort systems into those essential to safe service and those that are administrative.	"We should identify which systems are essential to safe service and which are merely administrative."	S5	Stakeholder opinion	System inventory. Not established in transcript.
ST-22	Health-data governance	Questions on collection, purpose, storage, access, retention and sharing.	"What personal or health data is collected, why it is used, where it is stored, who can access it, how long it is retained, and whether it is shared across borders or with vendors."	S5	Transcript statement (questions)	Whether the target holds health data; answers. Not established in transcript.

ID	Topic	Paraphrase	Exact quote	Stakeholder	Classification	Missing information
ST-23	Cyber evidence	Policies alone do not prove controls work.	"A policy document alone does not prove controls work."	S5	Stakeholder opinion	Test results, breach history, backup tests. Not established in transcript.
ST-24	Integration risk	Poor migration can interrupt workflows and compromise records.	"Poor migration or interoperability can interrupt workflows, make records unavailable, break interfaces, or compromise data integrity."	S5	Stakeholder opinion	Systems and integration approach. Not established in transcript.
ST-25	AI opportunity	AI upside is a hypothesis until key conditions are met.	"Any AI opportunity should be treated as a hypothesis until data rights, workflow fit, safety, oversight and measurable benefits are established."	S5	Hypothesis (so described by S5)	No AI use case described. Not established in transcript.
ST-26	Technology costs	List of cost items for the deal case.	"Cyber remediation, licence and vendor costs, data migration, interoperability, system validation where required, resilience improvements, and the time needed to integrate or replace platforms."	S5	Transcript statement (request)	Cost amounts. Not established in transcript.
ST-27	Reimbursement / payer exposure	If payer-dependent, scrutinise payer mix, rates, denials and collections.	"If revenue depends on payers, insurers or public reimbursement, examine payer mix, rates, denials, claims ageing and collection cycles."	S6	Transcript statement (conditional request)	Whether the target depends on payers. Not established in transcript.
ST-28	Revenue quality	Reconcile reported revenue to contracts, invoices, cash and activity.	"Reconcile reported revenue to contracts, invoices, cash collections and underlying activity."	S6	Transcript statement (request)	Reported revenue and financials. Not established in transcript.
ST-29	Hidden costs vs headline price	Remediation, upgrades, working capital, retention and integration may add cost.	"liabilities that are not obvious from the headline valuation."	S6	Stakeholder opinion	Headline price, valuation, quantified costs. Not established in transcript.
ST-30	Downside modelling	Show cash-flow and return impact of downside cases.	"The team should show the impact on cash flow and returns rather than just describe risks."	S6	Stakeholder opinion	Model and base case. Not established in transcript.
ST-31	Decision conditions	Recommendation may be conditional or paused if assumptions cannot be validated.	"If critical assumptions cannot be validated, the recommendation may need to be conditional or the decision paused."	S6	Stakeholder opinion	Client investment thresholds. Not established in transcript.
ST-32	Capacity to scale	Forecast demand does not show safe delivery capacity.	"A market forecast does not tell us whether the target can safely deliver the extra demand."	S7	Stakeholder opinion	Capacity, bottlenecks, workforce. Not established in transcript.
ST-33	Supply-chain risk	Single-source suppliers, lead times, cold chain, traceability and contingency are priorities if relevant.	"Identify single-source suppliers, long lead times, cold-chain or special storage needs if relevant, product traceability"	S7	Transcript statement (request)	Business type and suppliers. Not established in transcript.
ST-34	Operational data	Definitions and trends matter more than a snapshot.	"Definitions and historical trends matter more than a one-off snapshot."	S7	Stakeholder opinion	Operational data series. Not established in transcript.
ST-35	Integration disruption	Rapid change can disrupt availability and quality.	"Changing suppliers, systems, processes or leadership too quickly can disrupt availability, care workflows, service levels or quality."	S7	Stakeholder opinion	Integration plan; stop authority. Not established in transcript.

ID	Topic	Paraphrase	Exact quote	Stakeholder	Classification	Missing information
ST-36	Day 1 readiness	Artefacts needed before closing or Day 1.	"A critical-operations map, business continuity plan, escalation contacts, supplier and service dependency list, clear ownership of quality issues, and a phased integration roadmap."	S7	Transcript statement (request)	Whether any exist. Not established in transcript.
ST-37	Accessible vs large market	A large market is not necessarily accessible.	"Do not equate a large healthcare market with an accessible market."	S8	Stakeholder opinion	Countries, segments, local barriers. Not established in transcript.
ST-38	Key-person dependence	Relationships may depend on a founder or a few individuals.	"If relationships depend on one founder or a few individuals, we need to know whether those people will remain and whether the relationships are institutional or personal."	S8	Hypothesis (conditional risk)	Whether any such dependency exists. Not established in transcript.
ST-39	Post-close delays	Local steps can delay entry after closing.	"Local registration or licensing steps, procurement cycles, reimbursement decisions where applicable, onboarding requirements, customer approvals, language and service expectations, or gaps in local support."	S8	Stakeholder opinion	Local timelines. Not established in transcript.
ST-40	Customer trust on ownership change	S8 would ask customers and partners whether ownership change affects trust, contracts, service or purchasing.	"Would a change of ownership affect trust, contracts, service, or purchasing decisions?"	S8	Transcript statement (question)	No customer responses in transcript.
ST-41	Evidence discipline	Assumptions should rest on customer evidence.	"which assumptions are supported by customer evidence rather than management optimism."	S8	Stakeholder opinion	Customer evidence. Not established in transcript.

Appendix B: Stakeholders interviewed

ID	Stakeholder	Healthcare acquisition lens
S1	Chief Strategy Officer	Market entry rationale, strategic fit, alternatives, success measures
S2	Healthcare Commercial / Market Access Director	Demand, competitors, payer/provider purchasing, reimbursement or tender barriers
S3	Chief Medical Officer / Clinical & Product Quality Lead	Clinical value, patient safety, outcomes, complaints, product/service evidence
S4	Regulatory & Healthcare Compliance Director	Licences, approvals, inspections, quality findings, promotion and compliance exposure
S5	CIO / Health Data & Cybersecurity Lead	Health-data governance, interoperability, cyber risk, technology integration
S6	CFO / Healthcare Deal Finance Lead	Revenue quality, payer/customer mix, reimbursement, working capital, valuation, downside
S7	COO / Supply Chain & Service Delivery Lead	Capacity, critical suppliers, product availability/service continuity, operating readiness
S8	Country / Regional Market Lead	Local market access, customer trust, local competitors, talent, execution constraints

All interviews are labelled in the transcripts as fictional practice transcripts. The transcripts do not identify individuals. Interview dates are Not established in transcript.